MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 4 OF 28

MFT 704

Ethical and Legal Issues in Therapy

14 teaching weeks · 5 practicals · 1 worked case · 2 figures

SEMESTER ONE

1.71 units · Prerequisite: none

> “This course trains judgement, not just compliance.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 704 — Ethical and Legal Issues in Therapy

MSc Marriage and Family Therapy · Semester One · 1.71 Units · 14 teaching weeks · Prerequisite: none

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	**Ethical and Legal Issues in Therapy**	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. This course trains judgement, not just compliance. You will learn the codes, the law, and — more importantly — how to reason when the codes and the law point in different directions, when the family's culture conflicts with the institution's policy, and when your own values are in the room.

How to use this book

Every week presents a **standard**, a **case**, and a **decision**. Ethics is not memorised; it is practised. The weekly incidents in this book are drawn from the ordinary texture of clinical life — the secret, the subpoena, the gift, the neighbour, the unpaid fee — because those are the situations that actually end careers.

1. Course description

Family therapy multiplies ethical complexity. In individual work there are two parties (you and the client). In family work there may be six, with conflicting interests, unequal power, and secrets that each person believes belongs to them. Add a school, a child protection agency, a court, an insurance company, and a faith community, and the question "who is my client?" becomes the central professional question of the course.

The course combines **normative ethics** (codes, duties, principles), **law** (the parts a clinician must know and the parts a clinician must refer), and **professional formation** (the kind of clinician you are becoming).

2. Learning outcomes

#	Outcome	Assessed
LO1	Apply the major ethical codes and principles to relational cases	Exam, case analysis
LO2	Explain confidentiality and its limits in multiperson therapy	Exam
LO3	Conduct and document a risk assessment for suicide, self-harm, violence and abuse	Skills assessment
LO4	Manage secrets, dual relationships, gifts, fees and boundary dilemmas defensibly	Case analyses
LO5	Describe the legal framework for marriage, divorce, custody and child protection in Nigeria	Exam
LO6	Use a structured decision-making model and document the reasoning	Assignment
LO7	Recognise when to refer for legal advice, and how to do so	Viva

3. Assessment

Component	Weight	Due
Ethics incident analyses (4 × 600 words)	30%	Weeks 4, 6, 9, 11
Risk assessment observed skills (15 min)	20%	Week 10
Examination (case-based)	50%	Week 15

4. The fourteen-week plan

Week	Topic	Practical
1	Moral philosophy for clinicians: duties, consequences, virtues, care	Trolley variants in therapy
2	The codes: AAMFT, IAMFC, APA, national associations; informed consent	Consent form drafting
3	Confidentiality in couple and family therapy	Secrecy protocol design
4	Secrets I: the individual disclosure	Scripted incident
5	Secrets II: infidelity, paternity, HIV status	Complex case debate
6	Boundaries and multiple relationships	Boundary audit
7	Competence, supervision, and the duty to refer	Competence self-audit
8	Risk I: suicide and self-harm	**Observed risk assessment**
9	Risk II: intimate partner violence and child protection	Safety planning
10	Legal framework: marriage, divorce, custody, maintenance	Court-report exercise
11	Records, documentation, subpoenas, court testimony	Note-writing lab
12	Money, gifts, barter, and the business of therapy	Fee policy workshop
13	Ethics of research, assessment and diagnosis	Consent and deception cases
14	Integration: your ethical stance and decision model	Viva

Week 1 — Moral philosophy for clinicians

You do not need to become a philosopher, but you need to know which argument you are making when you say "that would be wrong".

Tradition	Central question	Therapy example
Deontology (duty)	What are my duties regardless of outcome?	Keeping confidentiality even when disclosure would be convenient
Consequentialism	What produces the best overall outcome?	Breaking confidentiality to prevent a serious harm
Virtue ethics	What would a person of good character do?	Refusing a gift that would create obligation

Tradition	Central question	Therapy example
Ethics of care	What does this relationship require of me?	Attending to the family's dependence on you
Justice / rights	Are power and resources fairly distributed?	Whose voice is missing from this room?

The four principles (Beauchamp & Childress) remain the most portable framework: **autonomy, beneficence, non-maleficence, justice**. In family work they collide constantly — respect for the father's autonomy may conflict with the mother's safety.

> **PRACTICAL**
> Take three ethical dilemmas from your own life (not clinical). For each, identify which tradition you used, and which you used to justify it afterwards.

Week 2 — The codes and informed consent

What codes do and do not do. Codes set a floor, not a ceiling. They are written to be enforceable, which means they are often silent on the hardest questions. They also vary by jurisdiction and by professional body; know which one governs you.

The core obligations across the major codes:

1. Competence and its limits
2. Informed consent (including consent for recording and for supervision)
3. Confidentiality and its limits
4. Multiple relationships
5. Sexual intimacy with clients and former clients (prohibited; time limits vary)
6. Fees and financial arrangements
7. Advertising and representation
8. Research ethics
9. Cultural competence and non-discrimination
10. Reporting obligations and legal compliance

Informed consent in family therapy is a document and a conversation. It should cover: who the client is; who is seen and in what combination; the confidentiality policy including secrets; recording and supervision; fees and cancellation; the limits of confidentiality (risk, abuse, court order); and the complaint route. Review it aloud in the first session, in the family's language, and record that you did.

> **PRACTICAL**
> Draft a one-page informed-consent document for couple therapy. Exchange with a peer and identify the three sentences most likely to be misunderstood by a family with limited literacy. Rewrite them.

Week 3 — Confidentiality in the multiperson system

There is no single confidentiality in family work; there is a **policy** you negotiate.

Three workable models.

Model	Rule	Advantages	Risks
No-secrets	Anything told individually may be shared with the couple	Simple, transparent	Individual may withhold important material
Full secrecy	Individual disclosures are kept private	Protects individual therapy	Creates a therapeutic secret that distorts the couple work
Selective (negotiated)	The therapist and client agree in advance what will be shared	Flexible and realistic	Requires skill and documentation

Most experienced couple therapists operate a **modified no-secrets policy**: they will not keep a secret that materially affects the couple's work, and they tell each partner this before the first individual session. This protects both the relationship and the therapist.

Documentation. Record the policy in the file, in the family's words if possible, and note the date it was explained. If a secret arises, record what you did and why.

> **CASE**
> During an individual session, a wife discloses that she has been having an affair for two years and has no intention of ending it or telling her husband. No one is at physical risk. Under a no-secrets policy, you tell her at the outset of the individual session that this cannot stay private if the couple work is to continue; you then help her decide how and when to disclose, with a deadline. Under a selective policy, you would negotiate what can be shared. What you must not do is carry the secret silently for months while conducting "couple therapy".

Week 4 — Secrets I: the individual disclosure

Classify before you respond. Three questions: Is anyone at risk? Does this information change the treatment I am providing? Is the client able to disclose it themselves, with support?

Type of secret	Typical response
Harmless (a surprise party)	Keep it; it is not clinically material
Clinically material, no risk (an affair; a debt; a diagnosis)	Negotiate disclosure with a deadline; do not collude
Risk to a third party (HIV status undisclosed; threats)	Assess; follow legal and public-health obligations; document

Type of secret	Typical response
Risk to a child or vulnerable adult	Report; inform; protect
Risk to self	Risk assessment; safety planning; break confidentiality as required

The clinical skill: refusing to be recruited. "I understand you are not ready to tell him. I cannot keep this and also work on your marriage with you, because I would be working with half the truth. Let us plan how you will tell him, and when."

The systemic reality. Secrets are not only information; they are structure. A secret held by one partner and the therapist creates a coalition — and the excluded partner often senses it, which damages the therapy's credibility.

Week 5 — Secrets II: infidelity, paternity, HIV status

Three cases that appear in every couple therapist's career, each with a different legal overlay.

Infidelity. Not a reportable act. The clinical tasks: stop the bleeding, contain the disclosure, assess for sexually transmitted infection risk, address the meaning of the affair (not only its fact), and decide with the couple whether disclosure to a third party (the affair partner's spouse) is required. That last question is a values question with safety implications; consult.

Paternity. A man may discover that a child he is raising is not biologically his. The therapist must not conceal information that affects a child's welfare, must not give legal advice, and must help the family decide who needs to know and in what sequence. Refer for legal advice on paternity, maintenance and custody; the emotional work is yours.

HIV and other communicable conditions. Confidentiality is strong, but public-health law may create duties where a partner is at identifiable risk and the client refuses disclosure. Know your jurisdiction's rule, consult, and document. Never disclose without advice.

> **DEBATE**
> "A therapist who keeps an affair secret for six months is colluding in the harm the affair causes." Argue for and against, then write your own policy in 200 words.

Week 6 — Boundaries and multiple relationships

Definition. A multiple relationship exists when you have another significant relationship with a client or someone closely associated with them. Not all are unethical; all require examination.

The test. (1) Could the second relationship impair your objectivity? (2) Could it exploit the power difference? (3) Would the client experience it as exploitative later? (4) Is it avoidable?

Common in small communities.

Situation	Risk	Defensible approach
Seeing a neighbour's family	Overhearing information; social contact	Discuss openly; agree how to behave at community events
Client is a fellow church member	Role confusion; spiritual authority	Clarify roles; consider referral if you hold a leadership role
A relative of your child's classmate	Dual loyalty	Disclose in supervision; transfer if the risk is unmanageable
Client asks you to be a character witness	Role change; advocacy	Decline; explain; refer to a professional who can assess
Gift-giving after a bereavement	Cultural obligation vs boundary	Accept small cultural gifts; document; discuss meaning
Trading therapy for services	Exploitation risk	Usually decline; if unavoidable, document value and rationale

Touch, self-disclosure, and home visits. Each is sometimes therapeutic and always requires a rationale you can articulate in supervision. The test is never "did it feel right?" but "whose need did it serve?"

> **PRACTICAL**
> Complete a boundary audit of your present life: community roles, family ties, business interests, social media. Identify the three situations most likely to produce a dual relationship in the next two years, and pre-decide your response.

Week 7 — Competence and referral

Competence has four dimensions: knowledge, skills, experience, and appropriateness (personal and cultural fit). You can be knowledgeable and inappropriate: a therapist with unprocessed trauma working intensively with trauma, or a young male therapist seeing a couple in a culture where his authority is contested.

The duty to refer arises when: the problem is outside your training; you lack supervision for it; your personal reactions would compromise care; or the client needs a different modality or medical input.

How to refer well. Name the reason without shaming ("this needs someone with specific training in X"); make a warm handover (a call, not just a name); offer a bridge session if needed; follow up once; and document. A referral is not abandonment.

> **SELF-AUDIT**
> List the ten presenting problems you are most likely to see. For each, rate your competence 1–5 and name the supervisor or service you would refer to. The two lowest ratings are your professional development plan for this year.

Week 8 — Risk I: suicide and self-harm

The standard. Ask directly, ask specifically, ask about means, and document. There is no evidence that asking about suicide increases risk, and substantial evidence that clinicians fail to ask.

A defensible risk assessment includes:

1. **Ideation** — frequency, intensity, duration, controllability.
2. **Plan** — specific method, place, time.
3. **Intent** — "Do you intend to act on this?"
4. **Means** — access to firearms, medication, pesticides, heights.
5. **Previous attempts** — the strongest single predictor.
6. **Protective factors** — reasons for living, responsibility to children, faith, support.
7. **Substance use and impulsivity.**
8. **Family history.**
9. **Current stressors and recent losses.**
10. **Disclosure and support network** — who can be told, who can watch.

Documentation. Record the questions you asked and the answers, your formulation of risk (low/moderate/ high), the plan, who was informed, and the review date. If you decide not to break confidentiality, record why.

In family work. A parent's suicidality, an adolescent's self-harm and a child's disclosure must be handled without turning the family session into an interrogation. Structure: see the person individually first, agree what will be shared, then facilitate the sharing.

> **PRACTICAL (OBSERVED)**
> Fifteen-minute suicide risk assessment role-play with a trained actor. You must ask all ten elements, formulate risk, and agree a safety plan. Pass/fail.

Week 9 — Risk II: intimate partner violence and child protection

Distinguish two patterns.

Situational couple violence	Coercive control
Mutual, low-level, escalates under stress	One partner systematically dominates
Fear is situational	Fear is chronic; surveillance, isolation, threats
Couple therapy may be appropriate after assessment	Couple therapy is contraindicated; risk increases
Safety planning individual	Specialist advocacy and protection

Screening. Screen each partner **separately**, at least once, using direct questions: "Has your partner ever hit, kicked, or pushed you? Has he/she ever made you afraid? Do you feel safe going home today?" Never screen in front of the partner.

Child protection. Know your jurisdiction's reporting duty. In Nigeria, the Child Rights Act (2003, adopted in many but not all states) and state child protection laws create duties and structures; the practical route is usually the state ministry responsible for social welfare, the police family unit, or a designated safeguarding lead. Document the concern, the advice received, and the action taken.

> **CAUTION**
> Conjoint sessions can escalate violence after the session. If there is any indication of coercive control, separate sessions and a safety plan precede any conjoint work. Write it down.

Week 10 — The legal framework a clinician must know

This section is **orientation, not legal advice**. Statutes and state practice vary; verify with a lawyer.

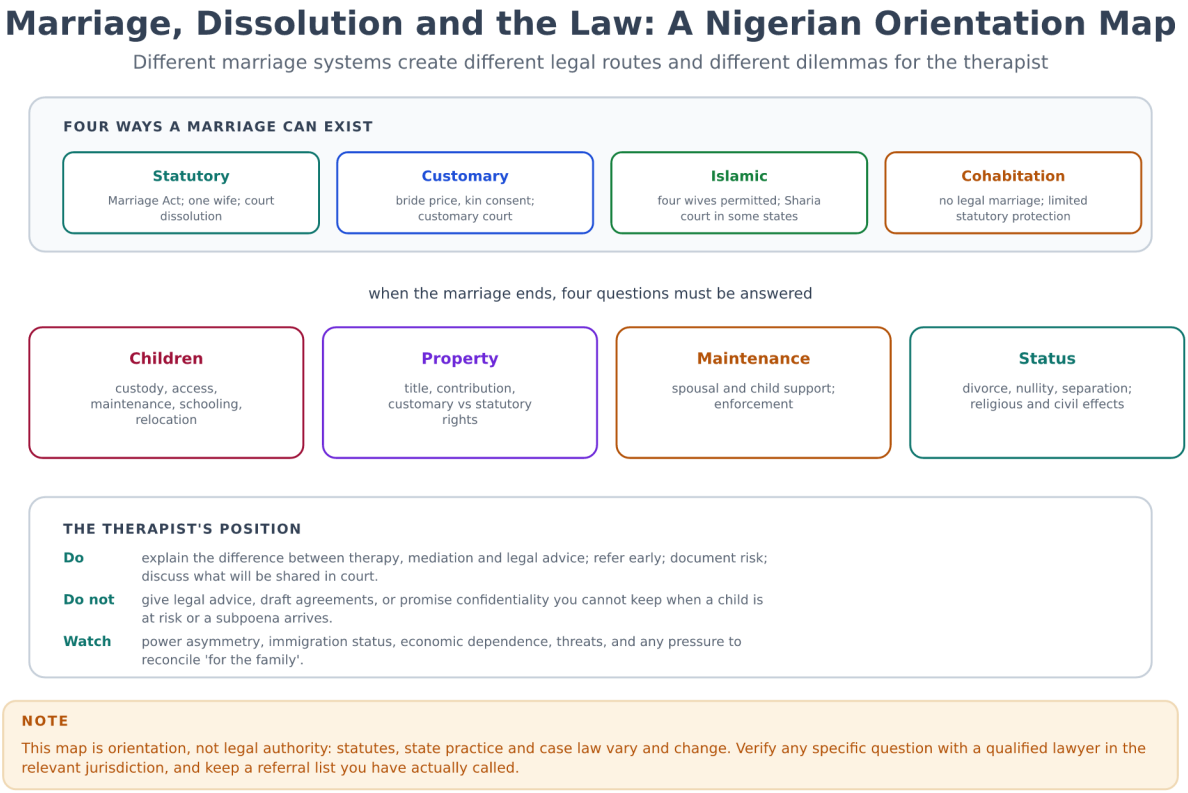


Figure 10.1 — Marriage systems and dissolution questions in Nigeria

Marriage in Nigeria exists in four overlapping forms: statutory (Marriage Act), customary, Islamic, and cohabitation without legal marriage. A family may be married under more than one system.

Dissolution. Statutory divorce requires a court petition (Matrimonial Causes Act, with the two-year rule and recognised grounds); customary and Islamic dissolutions follow their own processes and courts. The therapist's role is not to advise on grounds but to help the family clarify what they are deciding and to refer.

Children. Custody and access decisions are governed by the welfare principle — the child's best interests. Maintenance obligations exist under both statutory and customary law. The therapist may be asked for a report; a treating therapist should generally not conduct a forensic custody evaluation of their own client (role conflict).

Other relevant areas: the Child Rights Act; the Violence Against Persons (Prohibition) Act 2015 (applicable in the FCT and in states that have adopted it); mental health legislation in the states that have adopted the Mental Health Act 2021 framework; data protection obligations regarding client records.

Week 11 — Records, subpoenas and court

Write notes for three readers: your future self, a colleague taking over, and a court. Use factual, behavioural language; avoid speculation and avoid character judgements. Record: date, attendees, risk screen, interventions, plan, and who was told what.

If a subpoena or court order arrives: do not ignore it; do not respond immediately without advice; verify its validity; contact your supervisor, indemnity provider or lawyer; inform the client (unless prohibited); release only what is required, with a privilege log if advised.

Testifying. Your role is to report what you observed, not to advocate for a preferred outcome. Answer the question asked; do not volunteer; do not guess; say "I don't know" when you do not. Distinguish fact, clinical opinion and hypothesis.

> **PRACTICAL**
> Note-writing lab. Write a progress note for a session in which a risk was disclosed and a report to a school was agreed. Exchange; grade each other against the "three readers" standard.

Week 12 — Money, gifts and the business of therapy

Fees are clinical material. They carry meaning about worth, dependency, entitlement and shame. Have a written policy; state it at the start; apply it consistently; discuss non-payment rather than avoiding it.

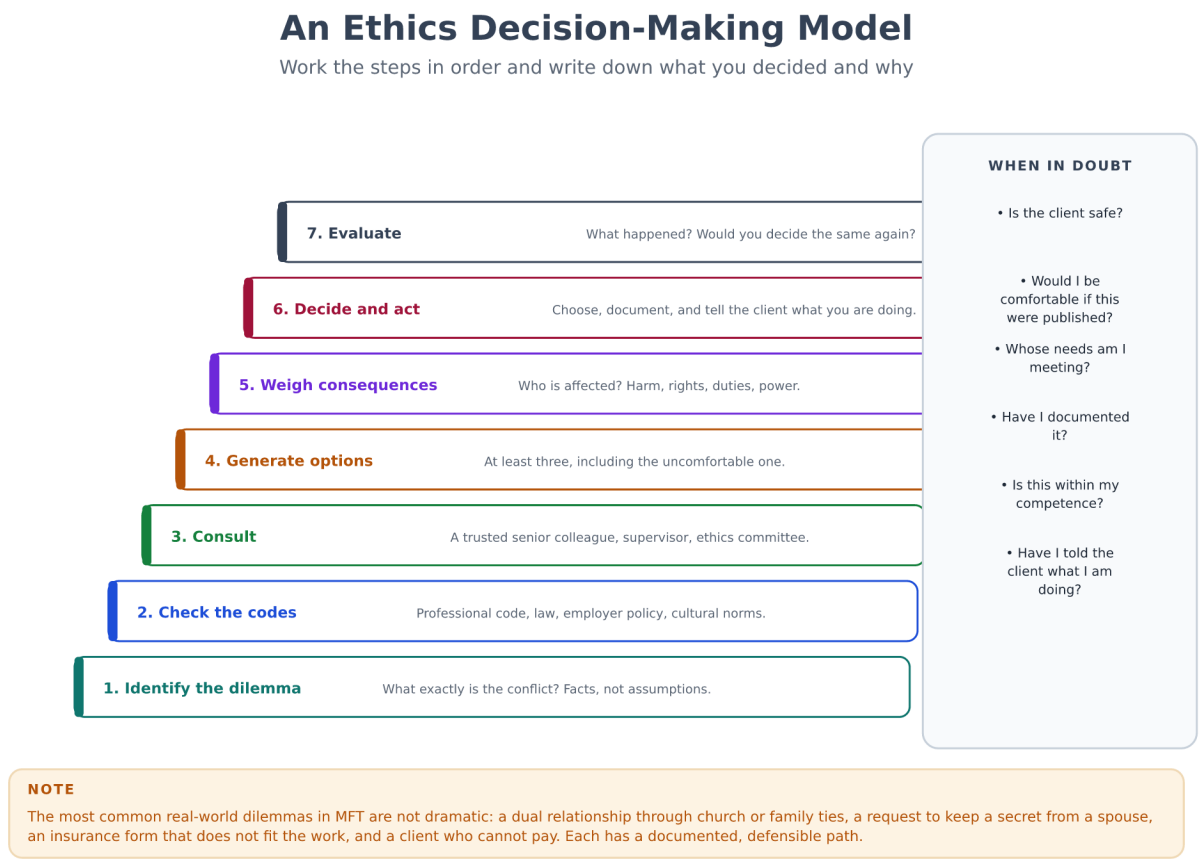
Refusing to reduce a fee sometimes ends therapy; never reducing a fee sometimes ends therapy. Both are true. Decide your sliding scale in advance and document the reason for each exception.

Gifts. Small cultural gifts may be accepted with documentation; expensive gifts should be discussed; gifts with strings should be declined. Ask, "What would it mean to you if I accepted / declined?"

Week 13 — Ethics of research, assessment and diagnosis

- **Informed consent** for research must be voluntary, capacity-appropriate and revocable; families, children and people in crisis require special care.
- **Deception** requires justification, debriefing, and ethics approval.
- **Diagnosis** carries consequences: access to care, stigma, insurance, custody. Diagnose what you can defend and describe what you cannot.
- **Assessment instruments** must be validated for the population and language; using an unvalidated instrument and acting on it is an ethical problem, not just a methodological one.

Week 14 — Integration: the decision model and your stance



NOTE

The most common real-world dilemmas in MFT are not dramatic: a dual relationship through church or family ties, a request to keep a secret from a spouse, an insurance form that does not fit the work, and a client who cannot pay. Each has a documented, defensible path.

Figure 14.1 — A seven-step decision-making model for ethical dilemmas

The seven steps. Identify the dilemma → check codes and law → consult → generate at least three options including the uncomfortable one → weigh consequences for each affected party → decide, act and document → evaluate afterwards.

The documentation habit. A defensible decision is not the same as a correct decision; it is one you can explain. Write the reasoning at the time, not after a complaint.

Your stance. Write 500 words: what you will never do; what you will always consult about; and where your personal values are most likely to distort your clinical judgement.

Glossary

Autonomy — the client's right to self-determination. **Beneficence / non-maleficence** — the duty to benefit and the duty not to harm. **Coercive control** — a pattern of domination, isolation and threat, distinct from situational violence. **Confidentiality** — the duty not to disclose information learned in a professional relationship. **Dual relationship** — a second significant relationship with a client or someone close to them. **Duty to warn / protect** — a legal obligation, in some jurisdictions, to act on a serious threat. **Informed consent** — voluntary agreement based on adequate information and capacity. **Mandated reporting** — a legal duty to report suspected abuse or neglect. **Privilege** — the client's legal right to prevent disclosure in court proceedings. **Subpoena** — a legal order requiring documents or testimony. **Welfare principle** — the rule that a child's best interests govern decisions about them.

Self-assessment

Multiple choice

1. In couple therapy, the most defensible approach to individual secrets is: (a) keep everything private (b) declare a policy in advance and apply it (c) tell the partner immediately (d) refuse individual sessions entirely

2. Asking a client directly about suicide: (a) increases risk (b) does not increase risk and is required practice (c) is only for psychiatrists (d) should be avoided in family sessions

3. Couple therapy with a couple in which there is coercive control is: (a) recommended (b) contraindicated until safety is established (c) harmless (d) the only legal option

4. The four principles of biomedical ethics are: (a) autonomy, beneficence, non-maleficence, justice (b) consent, capacity, confidentiality, competence (c) duty, utility, virtue, care (d) respect, honesty, integrity, courage

5. A treating therapist asked to conduct a custody evaluation for their own client should: (a) accept (b) accept if paid (c) decline because of role conflict (d) accept with supervision

6. The welfare principle means decisions about children are governed by: (a) parental preference (b) the child's best interests (c) the court's convenience (d) financial capacity

7. The strongest single predictor of a suicide attempt is: (a) diagnosis (b) previous attempt (c) gender (d) family income

8. A small cultural gift after a bereavement should usually be: (a) refused (b) accepted, discussed and documented (c) accepted silently (d) reported to the police

9. Screening for intimate partner violence should be done: (a) with the couple present (b) with each partner separately (c) by questionnaire only (d) only if asked

10. A defensible ethical decision is one that: (a) everyone agrees with (b) can be explained and documented (c) produces the best outcome (d) follows the client's preference

Answer key

1 b · 2 b · 3 b · 4 a · 5 c · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. State a confidentiality policy for couple therapy in six sentences a family could understand.
2. List the ten elements of a defensible suicide risk assessment.
3. Distinguish situational couple violence from coercive control, and state the treatment implication.
4. What should you do when you receive a subpoena? Five steps.
5. Why is "I know it is unethical but it was the right thing to do" an inadequate defence?

Essay (choose one, 2,000 words)

1. "The most difficult ethical problems in family therapy are not conflicts between right and wrong but between two rights." Discuss with three cases.
2. A father discloses that his 14-year-old daughter is self-harming and asks you not to tell her mother. Analyse the ethical, legal and clinical dimensions and set out your plan.
3. Critically examine the claim that the professional codes are culturally biased in favour of Western individualism.

Further reading

- American Association for Marriage and Family Therapy. (2015). Code of Ethics.
- Beauchamp, T. L., & Childress, J. F. (2019). Principles of Biomedical Ethics (8th ed.). OUP.
- Corey, G., Corey, M. S., & Corey, C. (2019). Issues and Ethics in the Helping Professions (10th ed.).
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- Wilcoxon, S. A., Remley, T. P., & Gladding, S. T. (2013). Ethical, Legal and Professional Issues in Counseling (4th ed.). Pearson.
- Child Rights Act (Nigeria), 2003; Violence Against Persons (Prohibition) Act, 2015.

Next: MFT 705 — Research Methods in Marriage and Family Therapy